

The Permitted Announcement

A website for Dr. Janine A. Morris’ private psychiatric practice — built as the one form of public communication the Medical Council expressly allows: an announcement of the practice and the services offered. Not an advertisement, and never mistakable for one.

PREPARED FOR

Dr. Janine A. Morris
B.Sc.(Hons) • MB.BS. • DM Psychiatry
Consultant Psychiatrist

PREPARED BY

[your name]
[practice / studio]

PRACTICE

Suite 2B, 3M Plaza
Kensington New Road & Baxter’s Road
St. Michael • three clinic days weekly

SCOPE

Design and build of a seven-page practice website, plus launch

JURISDICTIONS

Barbados (primary)
Jamaica (secondary)

VALID FOR

30 days from issue

You told me the one thing that decides the whole design

“The medical council does not allow doctors to advertise here. They can announce the opening of a practice and the services offered, but no advertising can be done.”

— DR. JANINE A. MORRIS, DISCOVERY CALL

Most web designers would hear that and quietly build you a small brochure site anyway, with a hero image, a call-to-action button and a testimonials carousel. That site would be an advertisement wearing a doctor’s coat, and it would put your registration at risk to win you very little.

Read the permission carefully and it is more generous than it first appears. You may announce **the practice** and **the services offered**. A website is simply the most complete, most permanent and most useful form that announcement can take — available at 2am to a worried relative, and at 11am to a GP with your name half-remembered.

So the design brief writes itself: **a reference, not a pitch**. Every page answers a question someone actually has. No page asks anyone to choose you.

The position the site has to serve

CLINICAL TIME

"I am only at the Suite three days a week. I don't work at the hospital."

Three clinic days, and no hospital switchboard behind you. The site carries the practical load a hospital would otherwise carry: where, when, and how to be seen.

CAPACITY

"I'm just starting my practice, so there is room for growth."

A new practice with open appointments. The site's job is to convert the interest you already generate, not to manufacture new demand.

WHERE PATIENTS COME FROM

"GP referrals, colleagues, hospital follow-ons, word of mouth. It varies."

Every one of those routes ends with somebody typing your name into a phone. Today that search returns nothing you control. That is the gap this site closes.

REACH

"I don't see patients online yet. I do also practice medicine in Jamaica."

Built so a telepsychiatry page and a Jamaica arm can be added later without a rebuild — and not published until each jurisdiction is confirmed.

REGULATORY POSITION

"No advertising. Announcement of the practice and the services offered is permitted."

The governing constraint. It sets what every page may say, and rules several standard website features out entirely.

What this website actually has to do

Three jobs, in order of how often they happen. Everything on the site earns its place by serving one of them.

JOB ONE – THE SEARCH

Be there when somebody looks you up by name

A patient has been given your name by their GP. A relative has heard it from a friend. Before anyone telephones a psychiatrist, they look them up — and what they find decides whether the call gets made. Right now that search returns a directory stub at best. It should return you: your qualifications, your Suite, your clinic days, stated plainly.

JOB TWO – THE REFERRAL

Let a referring doctor act in ninety seconds

A GP with a distressed patient in the room has moments to decide where to send them. They need four facts: does she take this, how do I send it, how long is the wait, and will I hear back. A page that answers those four questions is the single most commercially valuable thing on the site — and it is pure information, so it is entirely permitted.

JOB THREE – THE FIRST APPOINTMENT

Reduce the fear before the door opens

Seeing a psychiatrist for the first time is frightening in a way that seeing a cardiologist is not, and in Barbados it carries a stigma that keeps people away for years. A page that explains what the first appointment involves, how long it takes, and that it is confidential does real clinical good — and it does so without persuading anyone of anything.

Structure

Seven pages. Each one exists because somebody arrives needing exactly what is on it. Where a page carries a compliance risk, that is noted on the page itself rather than buried in a schedule.

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HOME · THE ANNOUNCEMENT

The practice, announced

The formal notice, given a permanent home: your name and post-nominals as they appear on your card, Consultant Psychiatrist, the Suite 2B address, clinic days, and how to make an appointment. Calm, uncrowded, and readable in one screen.

Name & post-nominals

Address

Clinic days

Appointment route

Crisis link

/about

ABOUT · STANDING

Dr. Janine A. Morris

Training, qualifications and registration, stated as a matter of record: B.Sc.(Hons), MB.BS., DM Psychiatry, registration in Barbados and Jamaica, and your areas of clinical interest. Facts a patient and a colleague both need, with no adjectives doing work they shouldn't.

Qualifications

Registrations

Clinical interests

Portrait

No superlatives, no years-of-experience claims framed as a selling point.

Services offered

The page the Council's wording most directly protects. A plain statement of what the practice provides — assessment, diagnosis, medication management, review and follow-up — and the presentations you see. Written as description, never as an offer.

Service list

Conditions seen

Age range

What is not offered

Final clinical wording is yours. I draft, you correct, nothing publishes without your sign-off.

For referring doctors

Written for a colleague, not a patient. What you accept and what you don't, how urgent cases are handled, the current wait time, exactly how to send a referral, and your commitment to write back after assessment. Printable to a single sheet, so it can sit in a GP's drawer.

Referral criteria

Urgency guidance

Current wait time

How to refer

Downloadable one-pager

What to expect

How long the first appointment lasts, what will be asked, what to bring, who may come with you, and how confidentiality works. The page that does the most to get a frightened person through the door — by informing them, which is not the same as persuading them.

Appointment length

What to bring

Confidentiality

Cancellation policy

Fees are discussed privately, not published. The page says so plainly.

/contact

CONTACT & LOCATION

Finding the Suite

Suite 2B, 3M Plaza, at the corner of Kensington New Road and Baxter's Road, St. Michael — with a map, parking guidance, the office line, clinic hours, and a short enquiry form that deliberately does not invite clinical detail.

Map & directions

Office line

Clinic hours

Minimal enquiry form

Worth deciding before launch: a dedicated practice email and a covered office line, rather than a personal mobile and a personal address.

/in-a-crisis

SAFETY • NON-NEGOTIABLE

If you need help now

Reachable from every page in one tap. Emergency and crisis numbers for Barbados, what to do out of hours, and a clear statement that the practice cannot respond to emergencies by email or web form. Someone will arrive on this site in genuine distress, and the site has a duty to them whether or not they ever become a patient.

Emergency numbers

Out-of-hours guidance

Persistent header link

Numbers to be supplied and verified by you — I will not publish a crisis number I have not had confirmed.

Built so that /telepsychiatry and a Jamaica page can be added later as an eighth and ninth page, once each jurisdiction's position on remote consultation is confirmed in writing. Nothing about the build forecloses that.

What goes on the site, and what never will

Your registration is worth immeasurably more than any traffic this site could earn. These are fixed terms of the build, not preferences — several are features I will decline to add if asked later.

THE SITE WILL	THE SITE WILL NOT
✓ Announce the practice and the services offered, factually ✓ State qualifications and registrations as a matter of record ✓ Explain conditions, pathways and what to expect ✓ Serve referring clinicians as professional information ✓ Carry crisis guidance on every page ✓ Go to you for sign-off, page by page, before launch	— Carry paid advertising, ad pixels or retargeting of any kind — Display testimonials, patient reviews or case stories — Publish consultation fees — Make comparative, superlative or outcome claims — Use urgency devices — countdowns, “limited slots”, pop-ups — Invite clinical detail through a web form

To be clear about the limits of my advice: I am a designer, not a regulatory adviser, and this plan rests on the position you described to me. Before launch I will put the full page-by-page wording to you in writing, and where there is any doubt I will ask that it be confirmed with the Medical Council or BAMP directly rather than assumed. **Jamaica is a separate jurisdiction** — no page addressing practice there goes live until that is confirmed independently. You hold final sign-off on every word.

Specification

ACCESSIBILITY

Usable by someone who is unwell

Built to WCAG 2.1 AA: real contrast, keyboard navigation, proper headings, text that resizes. This matters more than usual here — a meaningful share of your visitors will be anxious, depressed, medicated, elderly, or reading on a cracked phone in poor light. Accessibility is not a compliance checkbox on a psychiatry site; it is the point.

PERFORMANCE

Fast on a Caribbean mobile connection

Static pages, compressed images, no heavy frameworks, no third-party scripts that phone home. The whole site should load in under two seconds on mobile data, because that is how almost everyone will arrive.

PRIVACY

Built for the most sensitive category of data there is

The mere fact that somebody contacted a psychiatrist is sensitive personal data under Barbados' data-protection law. The enquiry form therefore collects the minimum — name, contact, preferred time — and explicitly asks people not to describe their symptoms. Submissions are encrypted in transit and delivered to a practice address, never to an unsecured inbox, and the site carries a privacy notice written for that standard.

FINDABILITY

Findable by name, without being promoted

Correct technical setup so that searching your name, or “psychiatrist Barbados”, returns the practice: structured data describing a medical practice, accurate address and hours, a clean sitemap, and consistent details across the directories where colleagues look. This is being listed correctly, not being advertised — but it is one of the items I would want confirmed with the Council before launch.

How we get there

WEEK 1

CONFIRM

Establish what may be said

- Regulatory position confirmed in writing before a word is drafted
 - Your service list, clinical scope and age range agreed
 - Domain, practice email and contact routes decided
 - Crisis numbers supplied and verified by you
-

WEEKS 2–3

WRITE

Content first, design second

- Every page drafted in full and sent to you for correction
 - Referral one-pager written with you, as the page that must be exact
 - Privacy notice and form wording drafted
 - Nothing designed until the words are signed off
-

WEEKS 3–4

DESIGN

An identity extended, not replaced

- Design built from the card you already have — the violet, the mark, and your line, *Confidential, Compassionate, Evidence-Based Care*
 - Home and one interior page presented for approval
 - Portrait photography arranged if you want one
-

WEEKS 4–5

BUILD

Assemble and test

- All seven pages built, responsive and accessible
- Form, encryption and delivery tested end to end
- Accessibility and performance audited against the spec above

WEEK 6

LAUNCH

Go live, and hand over

- Final page-by-page sign-off from you, in writing
- Domain, hosting, SSL and backups live
- Directory listings updated to match
- Handover session so you can edit text yourself, plus a dated file of everything published

INVESTMENT INDICATIVE

Fees

Figures are placeholders for you to set — the structure is what I am proposing.

INDICATIVE — AMOUNTS TO BE CONFIRMED

ITEM	INCLUDED	BASIS	FEE (BBD)
Website design & build WEEKS 1–6	Seven pages, copywriting, design, build, accessibility and performance audit, privacy notice, referral one-pager, launch and handover	Fixed project fee	\$0,000
Care plan OPTIONAL, FROM LAUNCH	Hosting, SSL, backups, security updates, wait-time and hours kept current, small text edits, annual compliance re-read	Monthly, cancellable	\$000 /mo
Telepsychiatry page LATER, IF CLEARED	Additional page and booking route, once both jurisdictions confirm their position on remote consultation	Quoted separately	—

Domain registration, hosting and any photography are passed through at cost and agreed with you in advance. Nothing is committed on your behalf without written approval.

What I need from you

- 01 **The Council's position in writing.** Confirmation of what a practice website may contain — particularly the services page and the directory listings — so the build rests on a document rather than a recollection.

- 02 **Your service list and clinical scope.** What you assess and treat, which presentations you don't take, and any age limits. The services and referral pages are only useful if this is exact.

- 03 **Crisis numbers, verified.** The emergency and out-of-hours numbers you want published. I will not put an unverified crisis number in front of someone in distress.

- 04 **Clinic hours and current wait time.** The two facts both patients and referring doctors look for first.

- 05 **How you want to be reached.** Your card currently routes enquiries to a personal mobile, WhatsApp, an office line and a personal email. A dedicated practice address and a covered office line read as more established and keep your own number private — and this is the one detail that is awkward to change after launch.

- 06 **Domain and existing assets.** Whether a domain is already registered, and the original artwork for the card — the mark, and the violet — so the site matches what you are already handing people.

Where I'd start

Week one, on its own: get the Council's position in writing and agree the service list. That costs you nothing but a letter and an hour, and it determines whether the rest of this is a six-week build or a shorter, plainer one.

I would rather find that out before you have paid for anything than after. If the answer comes back narrower than we expect, I will tell you so and reduce the scope and the fee to match.

ACCEPTED — DR. JANINE A. MORRIS · SIGNATURE · DATE

Confidential — prepared for the named recipient only.

Not regulatory or legal advice. All published material subject to the recipient's sign-off and, where relevant, confirmation from the Medical Council of Barbados, BAMP, and the corresponding authority in Jamaica.